

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Nodule		☒
Case is (circle): <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials: <u>RG</u> Date reviewed: <u>4/19/11</u>		
<u>hw</u> <u>4/25/11</u>		

UUID: CCE8E855-2582-4214-B9B1-25F0431065CB

TCGA-AR-A24Q-01A-PR

Redacted



Final Diagnosis

Breast, left, simple mastectomy: Invasive ductal carcinoma, Nottingham grade III (of III), is identified forming a 6.0 x 4.0 x 3.5 cm tumor (AJCC pT3) in the central-upper inner region of the breast. The deep and lateral margins of resection are negative for tumor. The closest margin is deep and is free by 1.0 cm. The overlying skin and nipple are not involved by carcinoma.

bilateral breast ca

Breast, right, simple mastectomy: Invasive ductal carcinoma, Nottingham grade III (of III), is identified forming a 1.7 x 1.3 x 1.0 cm tumor (AJCC pT1c) in the upper outer region of the breast. The deep and lateral margins of resection are negative for tumor. The closest margin is deep and is free by 2.8 cm. The overlying skin and nipple are negative for involvement by carcinoma.

Lymph nodes, left axillary sentinel, excision: Multiple (2) left axillary sentinel lymph nodes, both containing blue dye, are negative for metastatic carcinoma [(AJCC pN0 (i-)]. Immunohistochemical cytokeratin stain was performed on the paraffin embedded sentinel lymph node tissue and confirms the H&E impression.

Lymph nodes, right axillary sentinel, excision: Multiple (4) right axillary sentinel lymph nodes are negative for tumor [(AJCC pN0 (i-)]. A single lymph node (1A) contains blue dye. Immunohistochemical cytokeratin stains were performed on the paraffin embedded sentinel lymph node tissue and confirm the H&E impression.

Her-2/NEU has been ordered on paraffin-embedded tissue on the left breast.

ICD-O-3

carcinoma, infiltrating duct, NOS 8500/3

Site: breast, NOS C50.9 hw 4/25/11